

## CALCIUM LACTATE 300MG TABLET

Each tablet contains: -  
Calcium Lactate 300mg  
elemental calcium 39mg

### Product Description:

Round, white, plain tablet with single score on one side only.

### Pharmacology:

The major portion of absorption of calcium takes place in the proximal segments of the small bowel; in man, approximately one-third of the ingested calcium is absorbed. The absorption of calcium is, in part, a process of active transport.

There is a dynamic equilibrium between the calcium in the blood and that in the skeleton. Homeostasis is mainly regulated by the parathyroid hormone, by calcitonin, and by vitamin D. Excess of parathyroid hormone mobilises calcium from the bones and increases the concentration of calcium in blood and other extracellular fluid.

Parathyroid hormone increases the tubular reabsorption of calcium, but calcium excretion may be increased in hyperparathyroidism. Calcitonin exerts a rather rapid but only brief increase in uptake of calcium by bone and so lowers the concentration of calcium in the extracellular fluid.

Calcium is also mobilised by 1,25-dihydroxycholecalciferol, the active metabolite of vitamin D. These effects are largely interdependent. There is an inverse relationship between the concentration of calcium and phosphate in blood.

Calcium is excreted in the sweat, bile, pancreatic juice, saliva, urine, faeces and milk. Up to 400 mg is excreted in the urine daily; the amount is directly related to the excretion of sodium ions and is also increased by increased dietary magnesium. Urinary excretion of calcium decreases when increased amounts of phosphorus are ingested and increases when the phosphorus intake is reduced.

It has been demonstrated that two-third of the filtered calcium is reabsorbed in the proximal convoluted, 20 to 25% in the Loop of Henle and 10% in the distal convoluted.

### Indication:

It is indicated as dietary calcium supplement in:

- Growing adolescents
- Pregnant women
- Post-menopausal women threatened with osteoporosis
- Elderly persons who are bedridden and thus facing slow calcium loss from the bones

### Recommended Dosage:

As a dietary supplement, 1 to 3 tablets to be taken daily after meals.

### Route of Administration:

Oral

### Contraindications:

It is contraindicated in patients who are sensitive to any of its components and in patients with a history of renal calculi.

### Warnings and Precautions:

Certain dietary substance interfere with the absorption of calcium. These include oxalic acid (bran and whole cereals) and phosphorus (milk and other dairy products).

When calcium is administered in large amounts for long periods hypercalcaemia and hypercalciuria may result. This is most likely to occur in patients with hypoparathyroidism who may be receiving high doses of vitamin D.

Calcium salts should be given cautiously to patients with impaired renal function or a history of renal calculi. In patients prone to formation of calculi in the urinary tract, an increased fluid intake is recommended.

### Interactions with Other Medicaments:

Administration of corticosteroids may interfere with calcium absorption. Calcium enhances the effects of digitalis on the heart and may precipitate digitalis intoxication. Calcium salts reduce the absorption of tetracyclines.

### Side Effects:

When given by mouth, calcium salts may cause constipation. The other side-effects are due to hypercalcaemia. Symptoms of hypercalcaemia may include anorexia, thirst, nausea, vomiting, constipation, abdominal pain, muscle weakness, polyuria, drowsiness, confusion, bone pain due to demineralisation, nephrocalcinosis, loss of renal concentrating capacity, renal calculi and in severe cases, cardiac arrhythmias, coma, and cardiac arrest.

### Symptoms and Treatment of Overdose:

Overdosage of Calcium Lactate may result in hypercalcaemia.

Symptoms is as mentioned in 'Side-Effects'.

Treatment:

Calcium intake should be reduced to a minimum and large amounts of fluid given to combat dehydration and reduce renal deposition of calcium. In mild to moderate hypercalcaemia sodium phosphate may be given by mouth. In more severe cases, reduction of serum calcium concentrations has been achieved by the intravenous infusion of sodium phosphate, sodium sulphate, sodium chloride or sodium citrate.

**Storage Condition:** Store at or below 30°C. Protect from light and moisture.

**Pack Size:** Bottle pack: A bottle of 500 tablets.

Blister pack: A box of 3 x 10, 10 x 10, 50 x 10, and 100 x 10 tablets per strip.

**Shelf-life:** 3 years.

**FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.**

Manufacturer & Product Registration Holder:

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